



Laboratory | Digital X-Ray

Electric Traction(Weight) | E.C.G.

Patient Name : VIMAL MANE
Age & Sex : 44 Years & Female
Referred By : DR.GANESH JAGTAP
Lab Name : NIDAN PATHOLOGY LAB &
DIAGNOSTICS

Reg Date : 14/07/2026 05:15 PM
Report Date : 14/07/2026 05:32 PM
Reg No : 4746
Sample ID : 4746



HEMATOLOGY

Complete Blood Count (CBC)

Test Description	Result	Unit	Reference Range
Hemoglobin (Hb)	10.5	gms/dl	11 - 15
R.B.C Count	4.17	mil./cmm	3.8 - 4.8
Packed Cell Volume (HCT)	37.5	%	36 - 46
Mean Cell Volume (MCV)	89.93	fL	83 - 101
Mean Cell Haemoglobin (MCH)	25.18	pg	27 - 33
Mean Corpuscular Hb Conc. (MCHC)	28.00	%	32 - 38
RDW-CV	14.4	%	11.60 - 14
Total (WBC) Count	8300	cells/c.mm	4000 - 10000
Differential % WBCs count			
Neutrophil	74	%	45 - 75
Lymphocytes	20	%	20 - 40
Eosinophil	04	%	1 - 6
Monocytes	02	%	2 - 10
Basophils	00	%	0 - 2
Platelet Count	189000	cells/c.mm	150000 - 450000
Mean Platelet Volume (MPV)	10.1	fL	7.2 - 11.7
Peripheral Blood Smear			
RBC Morphology	Hypochromia		
WBC Morphology	Within Normal Limits		
Platelet Morphology	Adequate on smear		
Advice	Kindly correlate clinically & close follow up.		

SHUBHAM JAGTAP
(D.M.L.T.)

LAB TECHNICIAN



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BIOCHEMISTRY

RANDOM (BLOOD GLUCOSE LEVEL)

Test Description	Result	Unit	Reference Range
BLOOD SUGAR RANDOM	113.2	mg/dl	60 - 140
Urine Sugar (Random)	SNR		Absent
Urine Ketones (Random)	SNR		Absent
Method	GOD POD		

Interpretation:-

Elevated glucose levels (hyperglycemia) are most often encountered clinically in the setting of diabetes mellitus, but they may also occur with pancreatic neoplasms, hyperthyroidism, and adrenocortical dysfunction. Decreased glucose levels (hypoglycemia) may result from endogenous or exogenous insulin excess, prolonged starvation, or liver disease.

Fasting Glucose 2 HOURS PP Glucose Diagnosis

<110 <140 Normal

110 to 125 140 to 199 Pre Diabetes

>126 >200 Diabetes

Impaired glucose tolerance (IGT) fasting, means a person has an increased risk of developing type 2 diabetes but does not have it yet. A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes. IGT (2 hrs Post meal), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes. Note: Blood glucose level is maintained by a very complex integrated mechanism involving a critical interplay of the release of hormones and action of enzymes on key metabolic pathways. If postprandial glucose is lower than fasting glucose, it is termed as postprandial reactive hypoglycemia (PRH). The possible cause of PRH are high insulin sensitivity, exaggerated response of insulin and glucagon-like peptide 1, defects in counter-regulation, very lean individuals, anxious individuals, after massive weight reduction, women with lower body overweight physical activity prior test, hypoglycemic medication, deliberately eating less or eat a non-carbohydrate meal before testing.

Ref : American Diabetes association standards of medical care.

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CLINICAL PATHOLOGY

URINE ROUTINE

Test Description	Result	Unit	Reference Range
PHYSICAL EXAMINATION			
Quantity	20	ml	
Colour	Pale Yellow		Pale yellow/Yellow
Appearance	<u>Slightly Hazy</u>		Clear
Specific Gravity	<u>1.015</u>		1.005-1.030
pH	Acidic		Acidic
Deposit	Absent		Absent
CHEMICAL EXAMINATION			
Protein	<u>Trace</u>		Absent
Sugar	Absent		Absent
Ketones	Absent		Absent
Bile Salt	Absent		Absent
Bile Pigment	Absent		Absent
Urobilinogen	Normal		Normal
MICROSCOPIC EXAMINATION			
Pus cell	<u>10-12</u>	-	0-5
Epithelial Cells	4-6		Upto 5
Red Blood Cells	Absent		Absent
Casts	Absent		Absent
Crystals	Absent		Absent
Bacteria	Absent		Absent

END OF REPORT

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